

HUDSON VALLEY RADIOLOGY ASSOCIATES

LHR A LENOX HILL RADIOLOGY AFFILIATE

HVRA NEWBURGH
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Phone: (845) 565-[REDACTED]
Fax: (845) 565-3617

Exam requested by:
ARIEL PEDOEEM DO
377 BROADWAY
NEWBURGH NY [REDACTED]

Patient: [REDACTED]
Date of Birth: [REDACTED]
Phone: (704) 349-0802
[REDACTED]
Date of Exam: [REDACTED]

EXAM: DIGITAL DIAGNOSTIC LEFT BREAST CALLBACK MAMMOGRAM AND TOMOSYNTHESIS WITH CAD AND BREAST ULTRASOUND

HISTORY: The patient [REDACTED] demonstrated: left breast calcifications. Age: 46 years old.

COMPARISON: The present examination is compared to prior imaging studies dating back to 2021

MAMMOGRAM:

TECHNIQUE: Low-dose full-field digital left breast tomosynthesis examination was performed with tomosynthesis acquisitions and synthesized 2D reconstructed mammogram obtained. Magnification compression CC, magnification compression ML and full-field ML views were obtained. Computer-aided detection was applied.

FINDINGS:

BREAST COMPOSITION: There are scattered areas of fibroglandular density.

The calcifications and architectural distortion identified on examination of [REDACTED] are better defined on the additional views, are suspicious and histological evaluation is warranted via stereotactic core biopsy. An additional grouping of calcifications located approximately 2.5 cm medial and slightly posterior to the above calcifications is also noted. The second grouping of calcifications are, clustered, pleomorphic and therefore stereotactic core biopsy is warranted.

The remainder of the visualized left breast, on the additional views is unremarkable and no [REDACTED] architectural distortion, or additional foci of a suspicious calcifications are identified.

BREAST ULTRASOUND:

TECHNIQUE: Targeted left breast ultrasound was performed.

FINDINGS:

Left breast:
Retroareolar mild duct ectasia without nodule, mass or filling defect.
Retroareolar complex cyst/cluster of cysts-0.8 x 0.5 x 0.8 cm.

IMPRESSION:

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Tel: [REDACTED] - Fax: [REDACTED] - [REDACTED]

ACCREDITATIONS .AMERICAN INSTITUTE OF ULTRASOUND IN MEDICINE.FETAL MEDICINE FOUNDATION.ACR AND MQSA FOR MAMMOGRAPHY

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LHR A LENOX HILL RADIOLOGY AFFILIATE

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Continued: Page 2 of 2

Patient: [REDACTED]

Date of Birth: [REDACTED]

1. (2) groupings of mammographically suspicious calcifications in the left breast for which (2) site stereotactic biopsy is warranted.
2. The remainder of the unilateral left mammogram and targeted left breast ultrasound are unremarkable.

FOLLOW-UP: Stereotactic biopsy.

ASSESSMENT: BI-RADS Category 5: Highly suggestive of malignancy.

I personally discussed the above findings and recommendations with the patient [REDACTED] stereotactic-guided core biopsy was recommended. The patient [REDACTED] action. The above findings and recommendations will be faxed and called to the office of ARIEL Y PEDOEEM DO on [REDACTED] by our staff.

This examination should not preclude the clinical evaluation of a suspicious palpable abnormality.

As per the FDA requirements, a layman's letter has been generated and sent to your patient [REDACTED] recommendations of this breast imaging study. We have entered your patient [REDACTED] them when they are due for their next breast imaging exam.

Thank you for the opportunity to participate in the care of this patient.

STEVEN G REICHARD DO Bd Cert Radiologist - [REDACTED]
Physician [REDACTED]

FDA Site Name: [REDACTED]
320 ROBINSON AVE, NEWBURGH, NY [REDACTED]

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Tel: [REDACTED] - Fax: [REDACTED] - [REDACTED]

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Exam requested by:
ANDREI MUNZER MD
1200 NY RTE 300
NEWBURGH NY [REDACTED]

Patient: [REDACTED]
Date of Birth: [REDACTED]
Phone: (704) 349-0802
[REDACTED]
Date of Exam: [REDACTED]

EXAM: ULTRASOUND BREAST BILATERAL COMPLETE

HISTORY: The patient [REDACTED] supplemental screening evaluation of dense breasts.

TECHNIQUE: A bilateral breast ultrasound was performed with complete evaluation of the four quadrants, retroareolar regions, and axillae.

COMPARISON: [REDACTED]

FINDINGS:

In the right breast in the 12 o'clock position 5 cm from the nipple is a 4 mm cyst with low-level internal echoes which is new from prior.

There is an 8 mm cyst in the right breast 12 o'clock position 5 cm from the nipple stable from prior.

In the right breast at the 7 o'clock position is a 1.2 x 1.1 x 1 cm cyst. This demonstrates low-level internal echoes which may be due to its deep location.

In the right breast at the 10 o'clock position 6 cm the nipple is a 1.1 cm cyst.

In the left breast at the 4 o'clock position 4 cm the nipple is a 6 x 8 x 4 mm cyst.

No [REDACTED] in either breast. There is no [REDACTED]

IMPRESSION: Multiple cysts identified bilaterally including a new 4 mm cyst with low-level internal echoes in the right breast at 12 o'clock position 5 cm from the nipple as well as a new 1.4 cm cyst with low-level internal echoes in the right breast at the 10 o'clock position 6 and from the nipple. Six-month follow-up focal right breast sonography is suggested to ensure stability.

FOLLOW-UP: Follow-up imaging in 6 months.

ASSESSMENT: BI-RADS Category 3: Probably benign.

This examination should not preclude the clinical evaluation of a suspicious palpable abnormality.

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Tel: [REDACTED] - Fax: [REDACTED] - [REDACTED]

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Continued: Page 2 of 2

Patient: [REDACTED]

Date of Birth: [REDACTED]

Thank you for the opportunity to participate in the care of this patient.

ANAND E KAKKANATT MD Bd Cert Radiologist - [REDACTED]
Physician [REDACTED]

FDA Site Name: [REDACTED]
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ARIEL PEDOEEM DO
377 BROADWAY
NEWBURGH NY

Patient:
Date of Birth:
Phone: (704) 349-0802
Date of Exam:

EXAM: DIGITAL BILATERAL SCREENING MAMMOGRAM AND TOMOSYNTHESIS WITH CAD AND BREAST ULTRASOUND

HISTORY: The patient a screening mammogram.
Family history of breast cancer-maternal aunt.

CLINICAL BREAST EXAMINATION: The patient clinical breast exam.

COMPARISON: The present examination is compared to prior imaging studies dating back to 2021.

MAMMOGRAM:

TECHNIQUE: The following views were obtained digitally: bilateral craniocaudal, bilateral mediolateral oblique. Low-dose full-field digital breast tomosynthesis examination was performed with tomosynthesis acquisitions and synthesized 2D reconstructed mammogram. Examination was reviewed with Saige-Dx, an Artificial Intelligence Diagnostic Aid Software.

FINDINGS:

BREAST COMPOSITION: There are scattered areas of fibroglandular density.

In the left central to central upper breast middle 1/3rd there is an area of architectural distortion with calcifications for which additional mammographic evaluation and targeted left breast ultrasound is indicated.

Mammographically benign subcentimeter nodules are identified bilaterally.

The remainder of the left, as well as the right breast are unremarkable and no distortion, or significant calcifications are detected.

BREAST ULTRASOUND:

HISTORY: Supplemental screening evaluation.

TECHNIQUE: A bilateral breast ultrasound was performed with complete evaluation of the four quadrants, retroareolar region, and axilla.

FINDINGS:

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Continued: Page 2 of 2

Patient:
Date of Birth:

Right breast:

Flushing cysts are identified throughout the breasts.

10 o'clock 6 cm from nipple complex cyst/cluster of cysts-1.0 cm-new.

Left breast: Fluctuating cysts are identified.

No in either breast. No nodes are detected.

IMPRESSION:

1. Focus of architectural distortion and calcifications in the left breast for which additional mammographic evaluation and targeted left breast ultrasound is indicated.
2. The remainder of the bilateral mammography and bilateral breast ultrasound are unremarkable.

FOLLOW-UP: Additional imaging.

A diagnostic left mammogram with targeted ultrasound is recommended.

The Mammography Tracking Team will attempt to contact the patient .

ASSESSMENT: BI-RADS Category 0: Incomplete: Need additional imaging evaluation.

This examination should not preclude the clinical evaluation of a suspicious palpable abnormality.

As per the FDA requirements, a layman's letter has been generated and sent to your patient recommendations of this breast imaging study. We have entered your patient them when they are due for their next breast imaging exam.

Thank you for the opportunity to participate in the care of this patient.

STEVEN G REICHARD DO Bd Cert Radiologist -
Physician .

FDA Site Name: .
320 ROBINSON AVE, NEWBURGH, NY .

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Ph: 845.565.3664 - Fax:

Exam requested by:
MIGRATED REFERRING MD
MIGRATED RECORD

Patient:
Date of Birth:
Phone: (704) 349-0802

Date of Exam:

February 28, 2023

Andrei Munzer, MD
Crystal Run Healthcare
155 Crystal Run Rd
Middletown, NY
Tel# (845) 703-6999
Fax# 845/703-6265

RE: NADIA PATTERSON
DOB:
PT TEL# 704/349-0802
ACCT#:
STUDY DATE:

Dear Andrei Munzer, MD:

PROCEDURE: BILATERAL SCREENING 3D MAMMOGRAPHY

The patient clinical breast examination was one year ago.

Bilateral digital breast tomosynthesis was performed in the craniocaudal and mediolateral oblique positions. The examination was reviewed with the benefit of Computer-Assisted Detection.

Comparison is made to the prior examinations dating back to 2020.

Breast composition: There are scattered areas of fibroglandular density.

There is an apparent mass in the upper outer quadrant of the right breast. Additional imaging is recommended. No other suspicious masses, areas of architectural distortion or abnormal groups of microcalcifications are seen in either breast.

IMPRESSION:

Apparent mass in the upper quadrant of the right breast. A diagnostic unilateral right mammogram and targeted right breast sonogram is recommended for further evaluation.

Recommendation: Patient breast ultrasound.

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REPORT MIGRATED FROM RAMAPO/ORANGE RADIOLOGY LEGACY RIS SYSTEM



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Continued: Page 2 of 2

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(BI-RADS (R) Category 0-Incomplete: Needs Additional Imaging Evaluation)

The patient's information is entered into a mammogram tracking system with a target due date for the next mammogram.

AO 4:59 PM

PS Electronically Signed: Eric Trenkmann, MD 5:03 PMZ -

ET:PS
HVRA OF NEWBURGH

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***REPORT MIGRATED FROM RAMAPO/ORANGE RADIOLOGY LEGACY RIS
SYSTEM***